

CHRONIC OBSTRUCTIVE PULMONARY DISEASE (COPD): ROUTINE CARE

Ensure that a doctor confirms the diagnosis of COPD within 1 month and refer for spirometry if available. Refer the patient with newly diagnosed COPD for community health worker support.

Assess the patient with COPD

Assess	When to assess	Note
COPD symptoms	Every visit	- If patient has wheeze/tight chest and breathless at rest or while talking or respiratory rate ≥ 30, manage acute exacerbation ↪ 39. - Assess disease severity: if patient can walk as fast as others of same age, COPD is mild. If not, COPD is moderate or severe. - Investigate for TB only if patient has other TB symptoms like weight loss, night sweats, blood-stained sputum ↪ 92.
Other symptoms	Every visit	- Manage symptoms as on symptom pages. - If using inhaled corticosteroid and white patches on cheeks/gums/tongue/palate, oral candida likely ↪ 35. - If swelling in both legs, refer to doctor to consider heart failure. - If pain ≥ 4 weeks, assess and advise ↪ 61.
Adherence and inhaler technique	Every visit	Check adherence and that patient can use inhaler correctly ↪ 123. If not adherent, give adherence support ↪ 173.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↪ 143.
Palliative care	Every visit	If severe COPD with breathlessness at rest, > 3 hospital admissions for COPD in 1 year, heart failure or long term oxygen therapy needed ↪ 170.
CVD risk	At diagnosis	The patient with COPD is at increased risk of cardiovascular disease. Assess CVD risk ↪ 127.
Peak expiratory flow rate (PEFR)	- At diagnosis - If symptoms worsening - If change to medication at last visit	- Calculate % of predicted PEFR ↪ 124. - If 50-80%, COPD is moderate. - If $< 50\%$, COPD is severe.

Advise the patient with COPD

- If patient smokes, encourage to stop ↪ 141. Stopping smoking is the mainstay of COPD care. Support patient to change ↪ 177.
- Encourage the patient to take a walk daily and to increase activities of daily living like gardening, housework and using stairs instead of lifts.
- Inhaled corticosteroids can cause oral candida: advise patient to rinse and gargle after each dose of salmeterol/fluticasone.

Health for All

↪ 120

Treat the patient with COPD

- Give **influenza vaccine** 0.5mL IM yearly. Check that patient is up to date with COVID-19 vaccine.
- Give inhaled **salbutamol** 100-200mcg (1-2 puffs) 6-8 hourly, as needed.
- Before adjusting or starting treatment, ensure patient is adherent and knows how to use an inhaler and spacer correctly ↪ 123.
- If patient has **moderate** or **severe** COPD and not controlled on salbutamol alone, decide which treatment to add:
 - If COPD diagnosis confirmed on spirometry and < 2 exacerbations in past year: add inhaled **formoterol** 12mcg, 1 puff 12 hourly.
 - If spirometry not done, ≥ 2 exacerbations in past year or no better with formoterol: add inhaled **salmeterol/fluticasone**¹ 50/250mcg, 1 puff 12 hourly (stop formoterol if on it).
- If acute exacerbation was managed at this visit:
 - If patient received prednisone or hydrocortisone, continue **prednisone** 40mg daily for a total of 7 days.
 - If sputum increased or colour changed to yellow/green, give **amoxicillin** 500mg 8 hourly for 5 days. If severe penicillin allergy², give instead **doxycycline** 100mg 12 hourly for 5 days.

- If recent exacerbation, treatment adjustment, symptoms worse than usual or not coping as well as before, review monthly. Otherwise review 3-6 monthly.
- If no better with treatment after 3 months, discuss/refer.
 - Refer to Lung Unit to arrange long-term home oxygen therapy if patient is not smoking and still has moderate to severe symptoms (decreased oxygen saturations) despite treatment for ≥ 3 months.

¹If on lopinavir/ritonavir or atazanavir/ritonavir, avoid fluticasone and discuss/refer instead. ²History of anaphylaxis, urticaria or angioedema.